



PlatformTitle

MedicalReport: Chronic Lumbar Disc Herniation with Sciatica

PatientInformation

Name: ahmet kalumian

BirthDate: 2025-10-28

CaseID: #22

ClinicalDetails

Doctor: Dr. Alarkany

Date: 2026-04-28

Description: Patient presents with persistent radiating pain from the lower back down to the right calf. Diagnosed with L4-L5 disc protrusion. Pain is aggravated by prolonged sitting and forward bending.

PhysicalMeasurements

Height: 178 cm

Weight: 85 kg

BloodGroup: A+

HealthHistory

IsSmoker: No

ChronicDiseases: No

ActivityLevel: Moderate

SmartAnalysisTitle

Clinical Explanation:

The patient presents with persistent radiating pain from the lower back to the right calf, attributed to an L4-L5 disc protrusion. The pain exacerbates with prolonged sitting and forward bending, indicating a mechanical component to the pain. The patient's moderate activity level and absence of chronic diseases suggest a potential for recovery with appropriate rehabilitation strategies. The focus will be on pain management, improving range of motion, and strengthening the lumbar and pelvic regions.

Immediate Physiotherapy Recommendations:

A tailored physiotherapy program should include:

- Core stabilization exercises to support the lumbar spine.
- Flexibility exercises for the hamstrings and hip flexors to alleviate tension.
- Gradual introduction of aerobic conditioning to enhance overall fitness without exacerbating symptoms.

Precautions:

- Avoid prolonged sitting and forward bending activities that may aggravate symptoms.
- Monitor for any increase in pain or neurological symptoms during exercises.
- Ensure proper body mechanics during daily activities to prevent further injury.

Estimated Recovery Timeline:

With consistent physiotherapy and adherence to recommendations, a significant improvement may be expected within 6 to 12 weeks. However, full recovery may take longer depending on individual progress and adherence to the rehabilitation program.

Risk Assessment:

Potential risks include exacerbation of symptoms due to improper exercise execution or overexertion. Close monitoring of pain levels and functional abilities is essential to mitigate these risks.

Red Flags:

- Sudden increase in pain intensity.
- Development of bowel or bladder dysfunction.
- Progressive weakness in the lower extremities.
- Signs of cauda equina syndrome, including saddle anesthesia.

TestGroup: Range of Motion (ROM) Assessment

Test: Knee Flexion (Right)

Clinical AI Analysis

Normal Reference:
Normal knee flexion range for adults is typically between 0 to 135 degrees.

Clinical Interpretation:
The patient's knee flexion measurements indicate progressive improvement, moving from 95 degrees to 115 degrees over the assessment period. This suggests an increase in range of motion and potential reduction in pain-related limitations.

Progress Evaluation:
The trend shows a positive progression in knee flexion, indicating effective rehabilitation strategies and possibly reduced pain levels during movement.

Physiotherapy Focus:
Continue to emphasize knee flexion exercises, incorporating both active and passive range of motion activities. Monitor for any discomfort and adjust the intensity of exercises accordingly.

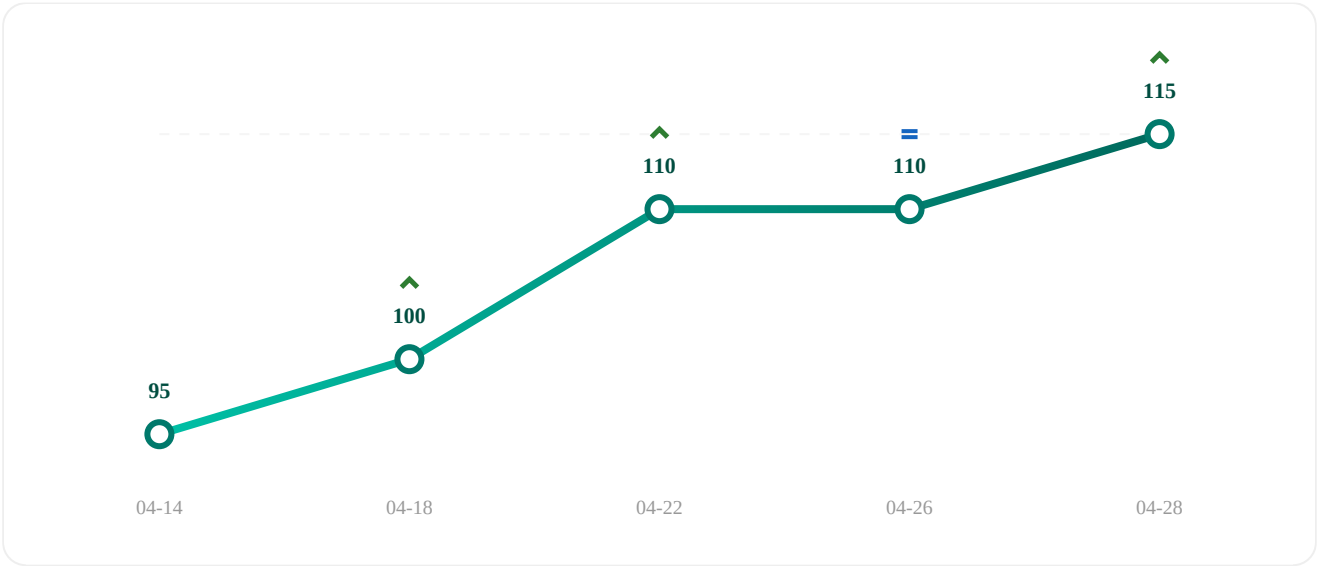
HIGHEST
115.0

LOWEST
95.0

AVERAGE
106.0

OVERALL PROGRESS

Increased by 21.1% ↑ Since first test (2026-04-14)



Date	Result
2026-04-14	95
2026-04-18	100
2026-04-22	110
2026-04-26	110
2026-04-28	115